Pennine Care
NHS Foundation Trust

DOCUMENT CONTROL	
Title:	**Inpatient Bed Management Policy – Adult and Older Peoples Service**
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Scope:	
This document applies to all Bed Managers, Night Managers and Staff working on Adult and Inpatient Wards.	
Purpose:	
The purpose of this document is to support and maintain safety of patients and support staff in making clinically appropriate decisions regarding admissions specifically when demand outstrips supply for inpatient services.	
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Other Trust documentation to which this policy relates (and when appropriate should be read in conjunction with):	
CL021	Section 136 MHA 1983 – Removal to a Place of Safety Policy
CO064	Privacy & Dignity Policy
Policy Associated Documents:	
TAD_CL125_01	Cross Border Activity Greater Manchester Mental Health
TAD_CL125_02	Procedure for Locating an Out of Area Bed
TAD_CL125_03	Mixed Sex Accommodation (MSA) Algorithm
TAD_CL125_04	OAPs definition and recording
TAD_CL125_05	Service User and Carer Access Standards - GM

TAD_CL125_06	Standards for Care Coordination for service users
TAD_CL125_07	Contact Names and Numbers within Core Hours Monday – Friday 09:00 – 17:00
TAD_CL125_08	Contact Names and Numbers outside Core Hours
Other external documentation/resources to which this policy relates:	
	Responsible Commissioner Guidance https://www.england.nhs.uk/who-pays/
CQC Regulations **This guideline supports the following CQC regulations:**	
Regulation 9	Person centred care
Regulation 10	Dignity and respect
Regulation 11	Consent
Regulation 12	Safe care and treatment
Regulation 13	Safeguarding service users from abuse and improper treatment
Regulation 14	Meeting nutritional and hydration needs
Regulation 17	Good governance
Regulation 18	Staffing
NICE Products: **This policy has been produced to reflect recommendations published in the following NICE products**	
None	

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1. INTRODUCTION

The Trust is committed to ensuring that where an individual requires a period of inpatient care this is provided in a clinically appropriate setting in a timely manner as close to home as possible.

Wherever possible patients should be admitted to a ward in their home locality (where they are registered with a GP and or resident. However, due to consistent pressures on bed occupancy, requirements with managing mixed sex accommodation and organic/functional presentation in Older Peoples Services, the following guidelines have been developed to assist acute and older people's services staff and on-call managers to effectively manage the inpatient capacity across the trust.

This policy has been developed to support and maintain safety of patients and support staff in making clinically appropriate decisions regarding admissions specifically when demand outstrips supply for inpatient services.

This policy is designed to support individual clinicians to ensure clinically appropriate admissions as per the policy. The policy describes what the options are – and especially what are not available, and these should be discussed with the senior manager or director on call.

Under no circumstances should the policy be breached without agreement with the Managing Director or identified deputy in hours and an Executive Director via gold on call out of hours, the proposal to breach should be accompanied by a clear clinical rationale, robust management plan and reported on the Trusts incident reporting system.

2. SCOPE OF POLICY

This policy refers to:

- All adult acute and older people's inpatient beds provided by Pennine Care NHS Trust.
- Any residents of one of the five Borough's within the Pennine Care footprint, aged 18 years and over (for residents under the age of 18 requiring an inpatient admission refer to the CAMHS inpatient admission policy in (TAD_CL125_02).
- Mixed Sex Algorithm

Patients presenting who are registered and registered in a borough/CCG outside of the PCFT footprint should ideally be admitted to a unit in their home provider (See TAD_CL125_01), where this isn't possible this policy applies to admission to PCFT units. See responsible commissioner guidance.

3. DEFINITION OF TERMS

For the purpose of this policy the following definitions will apply:

- **An available bed** is an empty bed, at the point that a request is made.

- **A safe leave bed** applies to a leave bed where the patient is not expected to return back within the next 24 hours during the week and 72 hours at weekends.
- **An unsafe leave bed** applies to a leave bed where the patient is expected back within the next 24 hours either due to an expected deterioration in their condition or an anticipated breakdown in their placement resulting in an imminent risk of readmission.
- **Safe AWOL bed** where an individual's location is known, and repatriation / representation is not expected imminently (within the next 24 hours)
- **Unsafe AWOL bed**: where an individual's location is not known, and repatriation / representation could occur at any point.
- **Transferring Borough** is the Borough the patient would normally receive inpatient services from (usually the locality where the patient is registered with a GP and/or resident).
- **Receiving Borough** is the Borough providing temporary inpatient care to the patient.
- **Core waking hours:** 9am-9pm
- **Night hours:** 9pm-9am
- **Core working hours:** Monday–Friday, 9am–5pm
- **Correctly assigned gender bed**: A bed where a sleeping breach would not occur.
- **Gender inappropriate bed**: A bed where an admission would constitute an MSA sleeping breach (see Mixed Sex Accommodation definition https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/215932/dh_121860.pdf)

4. GATE KEEPING ARRANGEMENTS

All requests for adult acute inpatient admissions need to be managed in accordance with the Trust's gate keeping policy through the relevant bed management team.

All gate keeping assessments must include verification of the patients GP and where possible residency, all assessments that conclude that an inpatient admission is required must be discussed with a senior medical practitioner (Consultant, SPR, Staff Grade, Associate Specialist) before a final decision is made to admit to an inpatient bed.

The borough where the individual who has been assessed as requiring a bed ordinarily resides is responsible for gatekeeping and identifying inpatient bed provision.

Bed management responsibilities are as follows:

Adult Acute and Older people inpatient beds

All Adult and Older People Bed requests are managed Monday – Sunday (Including Bank Holidays) 8am – 9pm by the Trust Wide Patient Flow Hub contactable on 0161 716 1207.

9pm – 8am All Adult and Older People Bed requests are managed by the Night Managers contactable by the switchboard

pcn-tr.patientflowhub@nhs.net

pcn-tr.nightmanager@nhs.net

Please note requests for inpatient beds are not accepted by email.

For individuals who are of No Fixed Abode at the point of gate keeping their GP and last known address will be utilised to determine which borough/CCG is responsible for their care. Where they have no registered GP or last known address then it is the responsibility of the borough where that patient presented to provide their care. This includes an NFA picked up by police on a S136/in police custody.

In all circumstances it remains the responsibility of the gate keeping borough to identify a suitable bed (adult, older adult, PICU) including those patients with no NHS Number/non-identifiable.

5. GENERAL BED MANAGEMENT PRINCIPLES

All admissions must be clinically appropriate (e.g., Patients are only admitted to an Older People's ward where they have a clinically appropriate presentation for that setting) and identified as unsuitable for available alternatives.

The individual responsible for bed management in a borough must explore the following options before looking for a bed outside of the individual's home borough:

a) Can a leave bed be used or created? (Identification of any other individuals who could be safely sent on leave following an appropriate assessment by the relevant practitioner or responsible Clinician)
b) Can individuals be transferred between wards in the borough to create a gender specific bed?
c) Can a bed be created through utilisation of the early discharge pathway through the HTT/HIT team?
d) Are there any individuals receiving inpatient care within the acute trust and still have a mental health bed allocated who are not due to be transferred back within the next 24 hours?
e) Are there any safe AWOL beds that could be utilised?
f) Are there any beds that are being held for a CTO recall bed, that have been vacant longer than 72 hours, and where it is known, there is no immediate plans to execute a Section 135(2) warrant.

All decisions in respect to the above must be considered in the context of the needs of all individuals that would be affected by these arrangements, including carers. If an individual is to be transferred between wards in order to create a bed this must only be undertaken within core waking hours, individuals should not be moved between beds or wards during the night (9pm-9am). Transfers can also only be undertaken with the individual's consent, (both detained and informal patients) they cannot be transferred to another ward to create a bed against their wishes.

If a bed cannot be identified once the above options have been explored, the individual responsible for bed management should contact the individual responsible for bed management in one of their neighbouring boroughs to identify an available bed. Boroughs within the same division must be contacted in the first instance; if a bed cannot be located, boroughs outside of the division should then be contacted or beds which are

subcontracted through the private sector. Subsequently neighbouring NHS Trusts should be contacted followed by private providers.

6. ADMISSION DOCUMENTATION PAPERWORK

All referrals for Acute Admissions must be made via secondary care professionals; all referrals will contain a standard set of information. This will include full clinical information and personal information on approved documentation.

The referral must also include a full risk assessment, undertaken within the last 24 hours of assessment, including any updates if the risks have changed since the full assessment took place. The referrer should ensure that a copy of the current risk assessment is available before contacting Bed Management team for a bed.

The minimum information required for admission is:

- Demographic information
- Presenting complaint
- Past and current mental health diagnosis and treatment (where indicated)
- Any significant medical issues or any special equipment required to meet their physical health needs
- Current and previous risks (including, offending history where known, drug and alcohol use)
- Social circumstances
- Safeguarding issues
- Expectations of service or treatment plan
- Gender Identity
- In addition to the above we must also ascertain the following-
- Reason for the admission
- Why the presentation can't be managed in the community
- Aim/expectation from the admission
- A suggested initial plan especially out of hours to avoid treatment delays

This information is required to seek the most appropriate bed in Trust, taking into consideration the individual ward's acuity, including potential Mixed Sex Accommodation risks. If there is no bed available in PCFT footprint, this information is also required to seek an Out of Area bed.

All referrals must have been through a gatekeeping process prior to be considered for bed identification.

7. USE OF OLDER PEOPLE'S BED

There will be no unplanned transfer of patients under 65 from an adult ward to an older people's ward.

For those service users under 65 who require admission where it is identified as clinically appropriate to admit to an older people's ward, this MUST be clearly indicated in the patient's care plan with the clinical rationale clearly documented.

Patients under 65 MUST ONLY be admitted to an older peoples ward as clinically appropriate (e.g., a patient with young onset dementia, people with a functional diagnosis with older people's needs e.g., frailty/high falls risk).

All transfer of patients under 65 to an older people's ward should be a planned transfer based on clinical need. The transfer will be to a ward where the patient will complete the episode of care as agreed with the MDT. The transfer should be agreed during working hours however, transfer can occur outside of working hours (Mon-Fri 9am–5pm).

All patients under 65 admitted to an older people's ward out of hours should be escalated to silver on call with a clear rationale for the admission with confirmation that this is included in the patients care plan.

Important Note:

There will be no admissions of adults with a functional illness to an older people's organic ward in any circumstances.

There will be no adults admitted where it is not clinically appropriate to an older people's ward.

It is the responsibility of the assessing practitioner seeking the bed to complete and forward all of the required documentation relating to the reason for admission, including an up-to-date risk assessment, gatekeeping information and care plan, to the borough where the bed is available prior to the admission of the individual.

Where possible the individual's notes should also accompany them to the ward where the bed has been identified. If the notes cannot be provided at the time of admission arrangements must be made to transfer them as soon as possible as a maximum 24 hours **(this only applies to wards within the trust footprint, notes should not be transferred outside of the organisation).**

8. ADMISSION TO SPECIALIST/SECURE/REHABILITATION BEDS

Specialist /Secure/Rehabilitation Beds should not be used to create capacity within acute services.

The use of Specialist /Secure/Rehabilitation Beds by patients with acute needs can compromise the care and recovery of those people on a rehabilitation pathway and is clinically not appropriate.

9. PATIENTS ADMITTED TO AN ACUTE PSYCHIATRIC WARD WITH A PRIMARY LEARNING DISABILITY AND OR AUTISM

All locality CCG mental health commissioners should be notified where a patient with a primary diagnosis of learning disability and or autism is admitted to an acute psychiatric ward to manage their presenting mental health problems. The following information should be provided on admission:

- Patient name
- NHS number
- Details of the patients MDT

10. UNSAFE LEAVE BEDS

Where a borough is declaring unsafe leave beds or is unable to facilitate an admission into an available bed due to concerns about high levels of clinical risk or staffing issues this should be discussed with the senior nurse on duty and the service manager or on-call senior manager.

Consideration will need to be given to requirements for increased levels of observation to facilitate an admission and the availability of staff to facilitate these. Admissions should not be agreed if they cannot be safely managed by the ward where the bed has been identified.

If a bed cannot be located in the Trust the individual responsible for bed management in the borough seeking the bed should contact neighbouring Trusts to ascertain if an alternative NHS bed can be utilised.

11. ADMISSIONS TO PRIVATE SECTOR BEDS

Once a private bed has been identified and an agreement has been given in principle that the private provider will accept the admission, the following process must be followed to agree funding:

Bury, Oldham & Rochdale

Monday – Friday 09:00-17:00 contact Sharon Wrigley, CCG Mental Health Case Manager Tel:07891 485749 Email: sharonwrigley@nhs.net

Out of hours contact senior manager on call for the relevant borough, who has delegated authority to agree funding on behalf of the CCG for up to 72 hours.

Tameside

Monday – Friday 09:00-17:00 contact Mental Health commissioner at Tameside & Glossop CCG

Out of hours contact senior manager on call for the relevant borough, who has delegated authority to agree funding on behalf of the CCG for up to 72 hours.

Stockport

Monday – Friday 09:00-17:00 contact Mental Health commissioner at Stockport CCG

Out of hours contact senior manager on call for the borough, who has delegated authority to agree funding on behalf of the CCG for up to 72 hours.

12. RECORDING OF OAPS

All patients registered to a GP/resident/NFA in the Pennine Care NHS Foundation Trust footprint, placed by PCFT in an inpatient facility outside of the Pennine Care NHS Foundation Trust footprint should be registered as an OAP as per the national and GM guidelines. (See TAD_CL125_04).

All national OAPs must be logged on the National Clinical Audit Platform by the bed management team (for further information contact the information department).

13. MIXED SEX ACCOMMODATION

All admissions to the inpatient wards should be to the correctly assigned gender bed, either designated single gender wards or designated single gender areas.

If the only bed available across the Trust footprint, at the time the admission is required, is a bed that is in an area assigned to a gender, follow the mixed sex algorithm (please see algorithm at TAD_CL125_03).

In order to prioritise patient safety and dignity admitting to a private bed should be considered before a breach admission is facilitated as per the algorithm. This should be considered in the context of the clinical presentation of the patient, the distance of the private bed, the risks presented and the expectation of a gender appropriate bed becoming available.

Where a patient is admitted to a gender inappropriate bed and a sleeping breach occurs:

- The patient must be placed on increased levels of observation to manage their safety, privacy and dignity for the entire period whilst occupying the wrong gender bed. This can be intermittent or constant observation based on clinical presentation and risk.
- A safety care plan must be in place to manage their privacy, dignity and safety needs whilst in this area.
- There should be clear documentation of the risk assessment and the decision to admit to a breach bed

Important Note:

Under no circumstances should a male be admitted to female only ward or vice versa.

Under no circumstances should a service user with a different gender be admitted to a multi-occupancy dormitory area.

All mixed sex accommodation breaches that relate to sleeping accommodation must be recorded via the collection tool available on tableau. http://penn-infossas/#/site/MHServices/views/MixedSexBreachCollectionTool/MixedSexBreachCollecti

onTool?:iid=1 . All admissions into an incorrect gender bed should be reported to the ward manager and service manager the next normal working day and monitored throughout each shift to identify the earliest opportunity to facilitate a gender appropriate bed.

All sleeping MSA breaches should be incident reported via the Trust's incident reporting system with the impact considering any psychological impact that may have occurred.

Every effort should be made to ensure that the service user is moved to a bed assigned to their correct gender as soon as possible after their admission at a maximum within 72 hours. Where transfer is not possible consider transfer to a private bed, if clinically appropriate.

14. TRANSGENDER PATIENTS

Transgender people (that is, people whose gender is not the same as, or does not sit comfortably with, the sex they were assigned at birth) are provided with legal protection against discrimination.

In addition, good practice requires that clinical responses be patient-centred, respectful and flexible towards all transgender people who do not meet these criteria but who live continuously or temporarily in a gender other than that which they were assigned at birth. This includes non-binary and gender fluid people.

General Key Points:

- Trans people should be accommodated according to the gender the person identifies with (the way they dress, and the name and pronouns that they currently use if staff are unsure of a patient's pronouns or gender, they should ask in a respectful way)
- This presentation may not always accord with the physical sex appearance of the chest or genitalia
- It does not depend upon the patient having a gender recognition certificate (GRC) or legal name change
- It applies to toilet and bathing facilities, trans patients should be afforded privacy as and when they need it. Please note most inpatient areas have en-suite showering facilities and this should be considered.
- The views of the trans person regarding where they wish to be accommodated should take precedence over those of family members where these are not the same.

All admissions of transgender patients should be assessed, and relevant risk management plans developed.

15. LOUNGES/S136 SUITES/STEM/SAFE HAVEN

Under no circumstances should these be used:

- Whilst a bed is located or made available for a patient
- For the primary reason to prevent a breach of A & E target

16. TRANSFER ARRANGEMENTS

Once an available bed has been located in another borough, the individual responsible for bed management in the transferring borough must contact the receiving borough to arrange the admission.

It is the responsibility of the borough seeking the bed to complete and forward documentation relating to the reason for admission and an up-to-date risk assessment to the borough where the bed is available prior to the transfer of the patient and where possible the patient's notes.

It is the responsibility of the transferring borough to make all arrangements for escorting and transporting the patient to the receiving borough and to ensure the patients carer/family are given all relevant information and contact details of the receiving inpatient facility.

17. REPATRIATION GUIDELINES

PCFT patients placed out of area within PCFT

Once a bed is identified to repatriate the patient to their home borough the individual responsible for bed management in the receiving borough will contact the individual responsible for bed management in the transferring borough to discuss repatriation arrangements.

There must be a clear handover provide to the receiving ward and the Consultant or RC.

Patients placed out of borough should ideally not be transferred to another borough which isn't their home borough.

The transferring borough should be cognisant of the whole patient journey and number of inter-borough transfers prior to transferring. If a patient is placed outside of their home borough and a further transfer is being proposed this must be communicated to the patient's home borough inpatient service.

It is the responsibility of the transferring borough to make all arrangements for escorting and transporting the patient back to their home borough and to keep the patient's carer/relatives and care coordinator advised of arrangements. Support with repatriation can be negotiated with the receiving borough if there is restricted capacity to facilitate this in the transferring borough.

The repatriation guidelines do not apply where planned arrangements are agreed between boroughs for patients to remain in the receiving borough for the duration of their admission.

PCFT patients placed in private/other NHS Beds

Where possible, priority must be given to repatriation of patients back from private beds over admission of other patients, however this decision should be based on presenting risk factors of individuals requiring a bed.

The bed manager should ensure that the consultant for the patient, and where relevant the patient's care coordinator, should be informed the next working day of an out of area placement occurring via the daily bed states.

Where the bed managers are made aware of PCFT patients placed out of the PCFT footprint repatriation should be facilitated as quickly as possible. A plan for timely repatriation should be made within 7 days where clinically appropriate.

Families should be kept appraised of the situation as per the GM service user and carer standards. See TAD_CL125_05. This should include clear information about the placement (location, type of placement, how the patient will be transferred and reassurance that the patient will be repatriated to the local area at the earliest opportunity).

Where bed managers are made aware that a patient has been admitted out of borough they should make contact where relevant with the patient's PCFT care team to inform them of the admission and placement location. The bed management team should keep regular (daily where possible) contact with the out of area provider and the patient's care coordinator to inform them of plans to repatriate.

Repatriation should be prioritised based on clinical need however ideally the most geographically remote patient should be prioritised where possible. There must be a clear handover provide to the receiving ward and the Consultant or RC.

Non PCFT patients placed OOA in a PCFT bed (e.g., Manchester patient in PCFT bed)

Attempts to repatriate to the patient's home provider on assessment should be attempted at the point of assessment and well documented. The responsible provider should be provided with all relevant documentation and where appropriate invited to undertake their own assessment/gatekeeping to inform a decision regarding next steps for the patient.

Where a decision to admit is reached the home provider should be contacted to be informed of the decision and to arrange admission ideally direct to the home provider.

Where the patients home provider/CCG (based on the responsible commissioner guidance) cannot facilitate an admission to their home inpatient/cannot support through an alternative to admission, a private bed should be identified and funded (including transport and conveyancing costs where relevant) by their responsible commissioner.

Where a provider/CCG refuses to support admission to the private sector, however, cannot support the patient through alternatives to admission then the issue should be escalated to senior managers in working hours and senior manager on call out of working hours. This should be clearly communicated to the responsible provider and commissioner and documented.

Important note:

No patient from another area should be placed in a private bed without prior discussion with their home provider/CCG.

In instances where no private beds are available the patient should be considered for admission to a PCFT bed and the responsible commissioner provider will be invoiced by the Trust.

All patients placed in a PCFT bed from another CCG area/Trust where there are no clear plans for repatriation within 7 days of admission (clinically appropriate) should be escalated to the relevant Associate Director with an audit trail of attempts to repatriate.

General principles

The bed managers are expected to work in collaboration to ensure that where possible the patient is repatriated to their home borough as soon as possible. In order to support this daily contact must be maintained between the divisional bed managers to review repatriation timescales.

The ability of carer and family in terms of maintaining contact with the patient should be considered as part of the transfer plan. Where possible the family should be supported to maintain contact as per the GM service user and carer standards (TAD_CL125_05).

There are occasions where there are clinical reasons to not repatriate patients to a home borough this should be explained to the relevant people including the patient, carers/families and the patient's home care team including consultant and care co-ordinator. The patient should be supported as per the GM care coordinator standards. (TAD_CL125_06)

18. HOME TREATMENT TEAM SUPPORT FOR ADULT SERVICE USERS

If an individual is admitted to an adult acute bed the HTT team from the individual's home borough should contact the ward where the individual has been admitted as soon as possible after admission and screen the individual for suitability for the supported discharge pathway.

The bed manager should inform the relevant HTT team where appropriate of a patient being admitted out of area. The gatekeeping process must consider HTT as an option before admission to an out of area bed.

19. KEY IMPORTANT NOTES

- There will be no admissions of adults with a functional illness to an older people's organic ward in any circumstances.
- There will be no adults admitted where it is not clinically appropriate to an older people's ward.
- Under no circumstances should a male be admitted to female only ward or vice versa.
- Under no circumstances should a service user with a different gender be admitted to a multi-occupancy dormitory area.
- No patient from another area should be placed in a private bed without prior discussion with their home provider/CCG.

- Under no circumstances should a lounge, safe haven or S136 suite be used:
  - Whilst a bed is located or made available for a patient
  - For the primary reason to prevent a breach of A&E target
- Where there are no beds in Trust, please refer to the escalation flowchart at the back of this policy.

20. EQUALITY IMPACT ANALYSIS

As part of its development, this document was analysed to consider / challenge and address any detrimental impact the policy may have on individuals and or groups protected by the Equality Act 2010. This analysis has been undertaken and recorded using the Trust's analysis tool, and appropriate measures will be taken to remove barriers and advance equality of opportunity in the delivery of this policy.

21. FREEDOM OF INFORMATION EXEMPTION ASSESSMENT

Under the Freedom of Information Act (2000) we are obliged to publish our policies on the Trust's website unless an exemption from disclosure applies. As part of its development, this policy was assessed to establish if it was suitable for publication under this legislation. The assessment aims to establish if disclosure of the policy could cause prejudice or harm to the Trust, or its staff, patients, or partners. This assessment has been undertaken using the Trust's Freedom of Information Exemption Guide and will be reviewed upon each policy review.

22. INFORMATION GOVERNANCE ASSESSMENT

This Policy has been analysed to ensure it is compliant with relevant information law and standards as in place at the time of approval and are consistent with the Trust's interpretation and implementation of information governance components such as data protection, confidentiality, consent, information risk, and records management.

Compliance will be reviewed against any changes to legislation / standards or at the next review of this document.

23. SAFEGUARDING

All staff have a responsibility to promote the welfare of children and young people and to protect adults at risk of abuse, enabling them to retain independence, well-being, dignity and choice. This requires a 'Think Family' approach as neither children, young people, adults and their families and carers exist or operate in isolation. Safeguarding is a range of activities undertaken by all of us supporting all adults and children's rights to be safe.

Where there are safeguarding concerns, all staff should act upon them in line with the Trust Safeguarding Families Policy and Local Safeguarding Partnership Procedures.

24. ANTI-FRAUD, BRIBERY AND CORRUPTION

The Trust is committed to reducing the level of fraud, bribery and corruption within the NHS and has adopted a Local Anti-Fraud, Bribery and Corruption Policy. Individuals should refer and adhere to this policy.

Concerns about fraud, bribery or corruption should be reported to the Trust's nominated Anti-Fraud Specialist using the contact details contained within the Local Anti-Fraud, Bribery and Corruption Policy. These details can also be found on the Trust's Counter Fraud intranet page.

Alternatively, concerns can be reported via the NHS Fraud and Corruption Reporting line on 0800 028 4060 or using the online report tool, www.reportnhsfraud.nhs.uk. All contacts are dealt with by experienced trained staff and anyone who wishes to remain anonymous may do so.

25. MONITORING

The effective application of this policy, including adherence to any standards identified within will be subject to monitoring using an appropriate methodology and design, such as clinical audit.

Monitoring will take place on a biannual basis and will be reportable to the Quality Group via the Clinical Effectiveness and Quality Improvement Team.

26. REVIEW

This policy will be reviewed three-yearly unless there is a need to do so prior to this; e.g. change in national guidance.

27. REFERENCES

Equality Act 2010

Freedom of Information Act (2000)